SAMPLE ANSWER · SPINAL CORD INJURY · CASE 2

# Sample 7-minute ISBAR: Natalie

Natalie Brooks · 44-year-old woman · Community occupational therapy home visit · about 1037 spoken words (≈ 7.4 min at 140 words per minute)

## I Introduction

*68 words*

"Hi, I'm the occupational therapy student on the home visit with Natalie Brooks for her wheelchair and cushion prescription. Natalie is 44 and has T10 AIS A paraplegia from a car crash six years ago. I'm calling you urgently because she has a worsening wound over her right sitting bone and signs that she may have a spreading infection, and she is declining to see a doctor today."

## S Situation

*145 words*

"During the visit I noticed a damp yellowish stain and an unpleasant smell on her old cushion cover. Natalie then told me she has had a sore over her right buttock for about three weeks, which she has been dressing herself using a mirror. She says it is bigger now and leaking.

"She has also felt 'flu-ish' for two days, had chills last night, woke up drenched in sweat, and her leg spasms are much worse than usual. She looks pale and was wrapped in a cardigan in a warm room.

"She has said she can't go to hospital because there is no one to mind the children and she has a work deadline on Friday. It's 2.20 pm, she wants to leave at quarter to three for school pick-up, and the supplier is waiting for me to sign off the trial wheelchair and cushion."

## B Background

*190 words*

"Natalie is a NIISQ participant. She is a single parent to Zara, 13, and Oscar, 10, and lives with them in a lowset house in Logan. She works from home full-time as an accountant and drives with hand controls. She has been independent with transfers, bladder and bowel care since rehab, and uses a manual wheelchair.

"The referral from her NIISQ support planner only mentioned that her cushion is worn and bottoming out. The wound isn't mentioned anywhere. She says it started after a run of long days at her desk.

"She also asked about her desk height and her right shoulder, which aches pushing up the driveway, and she has a NIISQ equipment form due Friday.

"Important gaps: I don't know her temperature, as I don't have a thermometer. I don't know the wound's size or depth, as I haven't seen it, and no health professional appears to have assessed it. I don't know when she last saw her GP, her medical history beyond the injury, or whether anyone else could collect the children. Her former partner is interstate and she says her mother isn't well enough to help."

## A Assessment

*287 words*

"My main concern is that this looks like a pressure injury that has become infected and may now be affecting her whole body. A wound over the ischial tuberosity that has grown over

three weeks, smells and is draining fits a pressure injury caused by long periods of sitting on a worn cushion. Chills, night sweats and feeling 'flu-ish' suggest the infection may be systemic. In someone with a complete injury at T10, increased spasticity is a recognised sign that something is wrong below the level of injury, such as infection. She can't feel the wound, so pain won't warn her, and her temperature control may also be affected, so she could be sicker than she seems. Deeper infection, including bone infection, and sepsis are real risks. That's why I think she needs medical review today, not next week.

"There's also an immediate safety issue with her plan to drive to school. If she's developing a serious infection, she could deteriorate, and her spasms are already strong enough to throw her legs off the footplate.

"Natalie is organised, friendly and transferring well, but that doesn't rule out serious infection. Her shoulder, desk height, the NIISQ form and the quote are all real needs, but none of them is urgent today. I also shouldn't sign off on a cushion while there is an open wound, because her seating needs will depend on the wound and medical advice.

"Her reasons for refusing are completely understandable. She is a sole parent with work pressure. I need to respect her right to make her own decisions, and I can't assess her capacity or decide on treatment. But she does need clear information about the risks, and her barriers may be solvable."

## R Recommendation

*347 words*

"First, I'd like your help right now, ideally by speaking with Natalie on the phone yourself. I'd explain calmly what I've noticed and why it worries me, and I'd recommend same-day medical review. That could be an urgent appointment with her GP this afternoon or an emergency department visit. If she becomes confused, very drowsy, breathless or develops shaking chills, or if she or I think she is deteriorating, I'd call triple zero.

"Second, I'd ask her to stop sitting on the wound now. Lying on her side or front would offload it while we arrange things. The trial cushion isn't a fix.

"Third, I'd help her work through the barriers. With her permission, I'd contact her NIISQ support planner and our team's social worker today to explore urgent childcare or support. I'd also ask whether the school's outside school hours care could take Zara and Oscar this afternoon, or whether a friend or another school parent could help. If she can't safely drive, transport needs to be arranged too.

"Fourth, I'd tell the supplier we won't finalise the prescription today. Any wound assessment, care and dressing advice should come from a nurse or doctor, so I'd recommend her GP or a community or wound care nurse follow up after medical review.

"For OT, once the infection is being treated and medical staff say she can sit, my principal short-term assessment would be a seating and pressure care assessment. That would include observing her sitting tolerance, pressure-relief routine and cushion condition, with pressure mapping if available. We'd then revisit the cushion and wheelchair prescription with her medical team's advice.

"Longer term, I'd use the COPM to understand her priorities as a working sole parent. That might cover desk set-up, scheduled pressure relief during long workdays, and her shoulder pain, which matters for wheelchair configuration and long-term upper limb preservation.

> "The International Pressure Injury Guideline and the NSQHS Comprehensive Care Standard both support early recognition and escalation of pressure injuries. My priority now is getting Natalie medically reviewed today, with practical support so she feels able to go."

## Coaching notes *not spoken*

### The immediate priority

An enlarging, smelly, draining ischial wound plus chills, sweats, malaise and increased spasms suggests an infected pressure injury with possible systemic infection. Natalie needs same-day medical review, offloading and practical help with childcare so she feels able to go.

### Distractors in this case

- **Natalie appears well, organised and transfers independently** — function doesn't rule out infection, and sensory loss can mask it.
- **The wheelchair quote and supplier's timeline** — prescribing over an open wound is premature.
- **School pick-up, the work deadline and the NIISQ form** — these are real barriers to solve, not reasons to delay care.
- **Shoulder pain and desk height** — valid OT goals for later.
- **"I'll ring the GP next week"** — too late given systemic signs.

### Missing information to clarify

- Temperature, pulse and general observations
- Wound size, depth and stage (assessed by nursing or medical staff)
- When she last saw her GP; medical history; medications
- Who else could support the children
- Her sitting hours and pressure-relief routine

### Useful references


- EPUAP/NPIAP/PPPIA, *International Guideline: Prevention and Treatment of Pressure Ulcers/Injuries* (2019)
- Australian Commission on Safety and Quality in Health Care, *NSQHS Standards – Comprehensive Care Standard*


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